



February 4, 2026

The Honorable Robert F. Kennedy Jr.
U.S. Department of Health and Human Services
Hubert H. Humphrey Building
200 Independence Ave., SW
Washington, DC 20001

Comments submitted electronically via www.regulations.gov

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Secretary Kennedy:

The National Multiple Sclerosis Society (Society) thanks the U.S. Department of Health and Human Services (HHS) for seeking feedback on how current HHS regulations affect the adoption and use of artificial intelligence (AI) in clinical care. As medical technology advances, the Society applauds HHS for working to ensure that the use of AI in clinical care, with proper guardrails, will allow all Americans to live their best lives.

Multiple Sclerosis (MS) is an unpredictable disease of the central nervous system. Currently, there is no cure. Symptoms vary from person to person and may include disabling fatigue, mobility challenges, cognitive changes, and vision issues. An estimated 1 million people live with MS in the United States. Early diagnosis and treatment are critical to minimize disability. Significant progress is being made to achieve a world free of MS.

The Society, founded in 1946, is the global leader of a growing movement dedicated to creating a world free of MS. The Society provides global leadership and funds research for a cure, drives change through advocacy, and provides programs and services to help people affected by MS live their best lives. To fulfill this mission, we fund cutting-edge research, drive change through advocacy, facilitate professional education, collaborate with MS organizations around the world, and provide services designed to help people affected by MS move their lives forward. Additionally, the Society sees itself as a partner to the government in many critical areas. While we advocate for the government's involvement in accelerating the discovery, development, and delivery of new treatments, we do it as an organization whose research investment has exceeded \$1.2 billion to date.

The Society acknowledges that HHS is one of the world's largest health research ecosystems. We applaud HHS for seeking input on ways it may invest in research and development to integrate AI into care delivery and create new, long-term market opportunities that improve the health and well-being of all Americans. We agree that AI will continue to transform and improve the healthcare landscape, if it is used in the appropriate setting, there is transparency about why it is being utilized, the goals of using AI are clearly outlined for all stakeholders, and the needs of patients and patient representatives are considered in the process. It is important to assess whether the use of AI introduces specific risks and harms, such as algorithms that could amplify errors and preexisting biases in the source data.

The Society focuses our response to this request for information (RFI) on specific question #9 - What challenges within healthcare do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care? Our response, outlined below, focuses on the following themes: patient-involved research and development of AI technologies, transparency and communication in AI development, and ensuring health equity in AI use.

AI has the potential to significantly improve MS clinical care by enhancing how clinical data are integrated, analyzed, and translated into actionable insights. AI can also support safety, effectiveness, and quality evaluations by integrating diverse data sources—such as genetic databases, clinical studies, and real-world evidence—to improve understanding of disease heterogeneity, progression, and treatment response. Applying AI to MS clinical care could lead to more personalized and effective treatment regimens, optimizing outcomes while minimizing unnecessary side effects.

However, the reliability and equity of AI tools hinge on the quality and representativeness of the data used to train these models. This underscores the critical importance of engaging individuals with lived experience of MS, especially from underrepresented communities, throughout the model development process. Their input ensures that AI tools are not only scientifically rigorous but also socially and ethically aligned with the diverse needs of people living with MS, thereby strengthening both the credibility and applicability of resulting innovations.

Patient-Involved Research and Development of AI Technologies

Individuals living with chronic conditions are the experts on their disease. These individuals, along with their carepartners and loved ones, hold a wealth of information that can inform the design of novel therapeutics and emerging technologies to support care delivery. The Society urges HHS and AI developers to meet with patients and their carepartners early and often throughout the development of AI technologies. Capturing the lived experiences of a diverse group of individuals living with chronic conditions, including MS, can ensure that the technologies developed are meaningful to patients and reflect their needs and concerns. This patient-involved research is essential for designing technologies that will benefit all patients and make them feel comfortable using them. The Society urges HHS to engage with the patient community by facilitating focus groups and patient-focused product development meetings. These meetings will not only allow for AI developers to gather input from those with lived experience but also build trust in the products as they were developed with the patient community.

Transparency and Communication in AI Development

The Society believes that for the adoption of AI to be successful, HHS and AI developers must be transparent and communicate to the patient community the goal of utilizing AI in clinical care, the scope of what these technologies can and cannot do, how the technology was developed, and how an individual's data is shared and secured. This information should be available on HHS's website.

We encourage HHS to work with community and patient groups so that educational efforts come from trusted sources. Additionally, the development of educational materials should consider how target audiences best receive information, include meaningful messages, and identify who is best suited to deliver the information. Focused offerings from peers in both formal and informal settings may help gain

trust in these technologies. Below, the Society offers suggestions on how best to engage with the patient community:

- Develop a range of educational offerings to reflect different ways people learn and receive information.
- Ensure all materials reflect health literacy principles and are written in plain language.
- Create educational materials that are multilingual, reflect cultural differences, include social determinants of health resources, and promote shared decision-making. Educational materials should include the current considerations and best practices of MS treatment so people can make informed decisions.
- Increase connections to diverse communities and use trusted sources, ambassadors, carepartners, and peers.

We advise HHS to consider a series of stakeholder feedback opportunities to allow the Department to share its plan for the strategic use of AI in clinical care and to gather stakeholder feedback to inform the process. Additionally, we recommend that HHS work cross-functionally with experts in academia and the patient community to routinely examine the algorithms and data that AI uses to ensure they do not cause harm or miss signals important to both HHS and patients.

In addition to providing transparent communications to all relevant stakeholders in accessible and stakeholder-specific language, the Society urges HHS to regularly engage with other government agencies as well as throughout the federal government, to ensure there is consistency with how AI is being used across the federal health and research agencies (e.g., FDA, NIH, CMS, CDC, etc.). Sharing learnings and best practices in AI use will help avoid duplicative efforts and create standards for uniform adoption of these technologies.

Ensuring Population Health in the Use of AI

Despite the promise of AI in accelerating biomedical innovation, its benefits must be equitably distributed to avoid perpetuating disparities in healthcare. The Society urges HHS to recognize concerns about algorithmic bias, incomplete datasets, and unintended consequences that could disproportionately affect certain patient populations. To address these challenges, the Society advocates for:

- The use of representative and high-quality data to build AI models that accurately reflect diverse patient populations.
- The establishment of a review process by an advisory panel of individuals with lived experience (e.g., a panel of individuals living with MS).
- The establishment of routine audits, validation processes, and accountability measures to detect and mitigate bias.
- The integration of population health principles into AI development, ensuring that technological advances do not exacerbate existing care gaps but instead drive meaningful improvements in health outcomes for all patients.

By prioritizing transparency, patient empowerment, and innovation in AI deployment, HHS can help foster accountability and build trust while ensuring that emerging technologies contribute to a more inclusive and effective healthcare system. The Society looks forward to continued collaboration with

HHS and other key stakeholders to shape policies that uphold these principles and advance the well-being of individuals living with MS.

Thank you again for your commitment to fostering innovation in medical technology development. As AI continues to become a major influence in healthcare, the Society looks forward to working with you to ensure that the voices of individuals with lived experience are heard in the development and refinement of these technologies. If you have any questions, please contact Nicole Boschi, Director of Regulatory Affairs, at nicole.boschi@nmss.org.

Sincerely,

A handwritten signature in black ink that reads "Steffany Stern". The script is fluid and cursive, with the first name "Steffany" and the last name "Stern" clearly legible.

Steffany Stern
Vice President, Advocacy
National Multiple Sclerosis Society